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Clinical Practice Guidelines for the management of Depression

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Participants of expert group on CPG for Depression

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INTRODUCTION

Depression is a common disorder, which often leads to poor quality of life and impaired role functioning. It is known to be a major contributor to the global burden of diseases and according to World Health Organization (WHO), depression is the fourth leading cause of disability worldwide and it is projected that by 2020, it will be the second most common leading cause of disability. Depression is also associated with high rates of suicidal behaviour and mortality. When depression occurs in the context of medical morbidity, it is associated with increased health care cost, longer duration of hospitalization, poor cooperation in treatment, poor treatment compliance and high rates of morbidity. Depression is also known to be associated with difficulties in role transitions (e.g., low education, high teen child-bearing, marital disruption, unstable employment) and poor role functioning (e.g., low marital quality, low work performance, low earnings). It is also reported to be a risk factor for the onset and persistence of a wide range of secondary disorders. Available data also suggests that between one-third and one-half of patients also experience recurrence of depressive episodes.

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ASSESSMENT AND EVALUATION (table-1)

Management of depression involves comprehensive assessment and proper establishment of diagnosis. The assessment must be based on detailed history, physical examination and mental state examinations.History must be obtained from all sources, especially the family. The diagnosis must be recorded as per the current diagnostic criteria.

Depression often presents with a combination of symptoms of depressed mood, loss of interest or pleasure, decreased energy and fatigue, reduced concentration and attention, reduced self-esteem and self-confidence, ideas of guilt and unworthiness, bleak and pessimistic views of the future, ideas or acts of self-harm or suicide, disturbed sleep and diminished appetite. Depending on the severity of depression some of these symptoms may be more marked and develop characteristic features that are widely regarded as having special clinical significance. These symptoms are known as somatic symptoms of depression and include symptoms of loss of interest or pleasure in activities that are normally enjoyable, lack of emotional reactivity to normally pleasurable surroundings and events, waking up in the morning 2 hours or more before the usual time, depression worse in the morning, objective evidence of definite psychomotor retardation or agitation (remarked on or reported by other people), marked loss of appetite, weight loss (often defined as 5% or more of body weight in the past month) and marked loss of libido. It is important to note that for the diagnosis of depressive disorder these symptoms need to be present for at least 2 weeks and need to be associated with psychosocial dysfunction.

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Table 1: Components of assessment and evaluation

Some of the patients with depression may present with

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